

UUID:66492A0C-48DA-4C18-A4A8-905282DCE5DB
TCGA-FD-A355-01A-PR

Redacted

Case#:

Sample

Gender: Male

DOB:

Race:

Report Date: .

Pathology Report:

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary	X	
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed by:	4/21/12	4/21/12

ICD-O-3

carcinoma, urothelial, NOS

8120/3

Surgical Pathology Report

Site:

path: bladder, NOS C67.9

CACF: bladder, trigone C67.0

FINAL PATHOLOGIC DIAGNOSIS

A. Left pelvic lymph nodes; dissection:

- Eight of twelve lymph nodes, positive for metastatic urothelial carcinoma (8/12).

- Extranodal tumor extension present.

4-9-12
RD

B. Right pelvic lymph nodes; dissection:

- Two of two lymph nodes, positive for metastatic urothelial carcinoma (2/2).

- Largest lymph nodal tumor measures 3.5 cm, with extranodal extension.

C. Right pelvic lymph nodes; dissection:

- Ten of eleven lymph nodes, positive for metastatic urothelial carcinoma (10/11).

- Extranodal tumor extension present.

D. Urinary bladder and prostate; cystoprostatectomy:

- Invasive high grade urothelial carcinoma, with mixed conventional, micropapillary features and squamous differentiation, see pathologic parameters.

- Tumor diffusely infiltrates the bladder wall, involving all bladder subsites, invades thru the muscularis propria, extends into the perivesical fat, involves bilateral seminal vesicles, and prostatic stroma.

- Tumor involves the margin at posterior perivesical soft tissue adjacent to prostate.

- Distal urethral margin, free of tumor.

E. Pre-sacral lymph nodes; dissection:

- One of two lymph nodes, positive for metastatic urothelial carcinoma (1/2).

- Extranodal tumor extension present.

F. Right common iliac lymph nodes; dissection:

- One of one lymph node, positive for metastatic urothelial carcinoma (1/1).

G. Left distal ureter; excision:

- Portion of ureter, no tumor.

H. Right distal ureter; excision:

- Portion of ureter with multiple microscopic tumor foci present within lymphovascular spaces in ureteral wall and periureteral fat.

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Invasive high grade urothelial carcinoma, with mixed conventional, micropapillary features and squamous differentiation.
2. Grade of tumor: High grade.
3. Depth of invasion: Extravesicular soft tissue and adjacent organs (prostatic stroma and bilateral seminal vesicles).
4. Tumor distribution: Diffuse, involving dome, right lateral wall, left lateral wall, trigone, anterior wall and posterior wall, with extravesical extension.
5. Ureteral margins: Tumor present in multiple lymphovascular spaces at the margin.
6. Distal urethral margin: Negative for tumor.
7. Soft tissue margin or serosa: Positive for tumor, at posterior perivesical soft tissue adjacent to prostate (slides D7 and D10).
8. Lymph nodes: Positive for tumor (22/28), largest lymph nodal metastasis 3.5 cm, with extranodal extension.
9. pTNM: pT4,N3,MX, R1.

Effective _____ this Checklist utilizes the _____ edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

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[] MD

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow, [], M.D., Ph.D.

Electronically Signed Out by [], MD

Clinical History:

This is a _____ year-old male with bladder cancer undergoing cystectomy.

Specimens Received:

A: Left pelvic lymph nodes

B: Right pelvic lymph nodes

C: Right pelvic lymph nodes
D: Bladder/ prostate
E: Pre-sacral lymph nodes
F: Right common iliac lymph nodes
G: Left distal ureter
H: Right distal ureter

Gross Description:

The specimens are received in eight containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "left pelvic lymph nodes". Received fresh and placed in formalin is a 5.5x5.5x1.5cm yellow lobulated fatty tissue. Multiple lymph nodes are identified, ranging in size from 0.3 cm to 5 cm. They are submitted as follows:

A1: 5 nodes
A2-3: 3 nodes in each cassette
A4-6: 1 node, bisected in each cassette
A7-9: 1 node, serially sectioned

B. The second container is additionally identified as, "right pelvic lymph nodes". Received fresh and placed in formalin is a 3.5 x 2.5x 2.5 cm yellow lobulated fatty tissue. 2 lymph nodes are identified, measuring 3.5x 2.5x 1.5 cm and 3.5x 1.0x 1.0 cm in size. The nodes are submitted as follows: B1-7: bigger node, B8-9: small node

C. The third container is additionally identified as, "right pelvic lymph nodes". Received fresh and placed in formalin is a 5.5x5.5x1.9 cm yellow lobulated fatty tissue. Multiple lymph nodes are identified, ranging in size from 0.3cm -6.6cm. They are submitted as follows:

C1-4: 2 nodes in each cassette
C5-6: 1 node, bisected
C7: 1 node, bisected
C8-9: 1 node, serially sectioned
C10-11: 1 node, serially sectioned

D. The fourth container is additionally identified as, "bladder/prostate". Received fresh and placed in formalin is a 433.1 gram, 12.5 x 9.5 x 6.5 cm cystoprostatectomy specimen with attached mesenteric fat. The bladder measures 7.5 x 7 x 6 cm. The right ureteral stump measures 2.1 cm and the left 2.0 cm, and both probe patent with resistance. The prostate measures 5.1 x 4.2 x 3.8 cm. The seminal vesicles and vas deferens are severely adhered to the dome of the bladder. The right seminal vesicle estimates 3.0 x 2.0 x 1.5 cm and the length of right vas deferens cannot be determined due to severe adhesions. The left seminal vesicle estimates 4.5 x 2 x 1.4 cm and the left vas deferens 11.5 cm long by 0.2-0.4 cm in diameter. The right half of the prostate and bladder is

inked blue and the left half is inked black. The prostate is opened anteriorly along the urethra and continued superiorly and laterally through the bladder. The prostate is serially sectioned from apex to base to reveal unremarkable, yellow parenchyma.

The bladder is diffusely involved by polypoid (0.1-0.5 cm) lesions, covering 9.0 by 6.5 cm surface area. Serial sectioning reveals tumor invasion into the adipose tissue to a depth of 2.5 5 cm, abutting the inked margin. Right seminal vesicle is involved by tumor. No normal mucosa is present.

Gross photographs are taken.

Representative sections are submitted as follows:

D1: Left ureter margin
D2: Right ureter margin
D3: Urethral margin
D4: Prostate apex
D5-8: Representative prostate through verumontanum
D9-12: Representative prostate above verumontanum
D13-14: Right and left Seminal vesicles and vasa deferentia
D15-17: Multiple surface polypoid lesions
D18-29: Deepest invasion (D18-20: One full thickness section involving seminal vesicle, D21: Tumor involving serosa, D22 and D23: Tumor involving fat, D24-27: One full-thickness section tumor involving fat, D28-29: One full-thickness section tumor involving fat)

E. The fifth container is additionally identified as, "pre-sacral lymph nodes". Received fresh and placed in formalin are 4 pieces of nodular tissue. 3 of them are submitted in E1, the fourth one is bisected and submitted in E2. Remaining specimen is entirely submitted in E3.

F. The sixth container is additionally identified as, "right common ileac". Received fresh and placed in formalin are two nodules, measuring 2cm and 1 cm respectively. The nodes are bisected and submitted in F1 and F2.

G. The seventh container is additionally identified as, "left distal ureter, stitch on nonmargin side". Received fresh and placed in formalin is a 0.4cm in length by 1.0 cm internal circumference segment of ureter, which is received opened longitudinally. A stitch is present at one side of the specimen, designating the non-margin side. The margin side is inked blue. The specimen is sectioned longitudinally and entirely submitted in G1.

H. The eighth container is additionally identified as, "right distal ureter, stitch on margin side". Received fresh and placed in formalin is a 0.9 cm long by 0.6 cm in diameter segment of ureter, with a stitch designating the marginal side. This side is inked blue. The specimen is opened longitudinally and no

gross lesions are identified. The ureter is entirely submitted as H1.

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[], M.D., Ph.D.